SUMMIT IMAGING CENTER

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Diagnostic Radiology Report
FINAL

PATIENT		DOB / SEX	MRN
DOE, JOHN		08/14/1991 · M	SIC-0093317
ACCESSION	EXAM DATE / TIME	EXAM	
SIC-MR-26-031307	03/13/2026 08:40	MRI CERVICAL SPINE WITHOUT CONTRAST (CPT 72141)	
REFERRING PHYSICIAN		READING RADIOLOGIST	
Alana Reyes, MD · Lakeside Orthopedics · Fax (555) 010-6501		Helen Park, MD	

CLINICAL HISTORY: MVA, date of injury 02/16/2026, neck pain with right arm numbness.

TECHNIQUE: Multiplanar, multisequence MR imaging of the cervical spine was performed on a 1.5 T system without intravenous contrast: sagittal T1, T2 and STIR; axial T2 and gradient-echo from C2 to T1.

COMPARISON: None available.

FINDINGS:

Alignment: Mild straightening of the normal cervical lordosis. No listhesis.
Vertebral bodies: Normal height and marrow signal. No fracture, bone marrow edema or destructive lesion on STIR.
Cord: Normal caliber and signal from the cervicomedullary junction through T1. No syrinx.
Craniocervical junction and prevertebral soft tissues: Unremarkable.

Level	Disc / canal / foramina
C2-C3	Normal disc height and hydration. No herniation. Canal and foramina patent.
C3-C4	Normal disc height and hydration. No herniation. Canal and foramina patent.
C4-C5	Normal disc height and hydration. No herniation. Facets normal. Canal and foramina patent.
C5-C6	**Right paracentral disc herniation (protrusion), 4 mm in AP dimension, with high-T2 signal at the posterior annulus. The herniation contacts the ventral right C6 nerve root in the lateral recess and causes mild right foraminal narrowing.** Mild effacement of the ventral thecal sac without cord contact. Left foramen patent. Disc height preserved; no endplate osteophytes; facets normal.
C6-C7	Normal disc height and hydration. No herniation. Canal and foramina patent.
C7-T1	Normal. Canal and foramina patent.

IMPRESSION:

1. C5-C6 right paracentral disc herniation with mild right foraminal narrowing, contacting the ventral right C6 nerve root, correlating with the stated right arm symptoms. Annular high-intensity signal at this level.
2. Disc height preserved at C5-C6; no significant degenerative change at the adjacent levels.
3. No fracture, ligamentous signal abnormality or cord signal change.

Helen Park, MD

Helen Park, MD, Diagnostic Radiology · electronically signed 03/13/2026 14:27

Report transmitted to Lakeside Orthopedics by fax 03/13/2026 14:31. Images available on request (CD/DVD). This report contains confidential health information.